

CHILDREN'S DRAWINGS

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Abstract—Children's drawings have been used in clinical interviews to provide direction for recollection and memory of events. Drawings encourage the retrieval of experience in the motoric, visual, and auditory recall. The drawing itself gives an expression of motor sensory discharge; the objects in the drawing help with the perceptual cues that are remembered; the cognitive dimensions are represented in the organization, interpersonal patterns, and verbal discussion of the picture. The premise of this paper is that children's drawings are useful as an associative tool for assessing and accessing traumatic memories.

Key Words—Children's drawings, Associative tool, Event drawings.

INTRODUCTION

THERE IS A strong tradition in the clinical use of children's drawings for projective testing and treatment interventions. Children's drawings are also helpful in assessing child sexual abuse. As stated in the position statement of the American Academy of Child and Adolescent Psychiatry of Guidelines for the Clinical Evaluation of Child and Adolescent Sexual Abuse (1988), "the usefulness of drawings lies in the affect and information they elicit, and certain findings which may be suggestive of sexual abuse such as depiction of genitalia or avoidance of sexual features altogether." Art is a process by which thoughts, feelings, behaviors, and relationships can be translated into concrete images. This paper reviews the literature on projective drawing tests and child sexual abuse, and reviews studies from the event drawing series.

LITERATURE REVIEW

The contributions made to psychometrics and to the psychology of child development by Florence Goodenough (1926) through her pioneering efforts in measuring children's intelligence by drawings have been integrated extensively in clinical pediatric and child health practice. Dale B. Harris (1963) re-evaluated Goodenough's method, attempting to extend it to older groups, and added two alternative tests of a drawing of a woman, and a drawing of the self. Harris' research replaced the notion of intelligence with the idea of conceptual maturity. This change focused away from the notion of unitary intelligence and permitted consideration of children's concepts of the human figure as an index of their general concepts.

During the early phase of the use of projective drawings as measurements of intellectual

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maturity, it became apparent to many clinicians that certain drawing tasks tended to tap personality factors in addition to intellectual and conceptual capabilities (Hammer, 1980). Although Goodenough's test was not intended to evaluate children emotionally or in the diagnosis of psychopathology, she, too, had observed the possibility of analyzing drawings for these purposes. She felt that children draw from the world they know rather than the world they see. In other words, a child, in drawing an object, projects his or her own experiences onto the drawing surface (Schildkrout & Shenker, 1972).

Studies were developed to evaluate emotional and developmental indicators of children's drawings. Human figure drawings were identified as sensitive indicators of psychopathology, reflective of disturbance of the body image of the child's self-concept and attitudes toward him/herself, of concerns about sexuality and bodily functions (Koppitz, 1968), and as capable of offering insights into a child's interpersonal relationships which other psychological techniques failed to provide (Burns & Kaufman, 1972). The nonintrusive, indirect approach of the drawing experience seemed to enable the child to reveal information to the examiner which would be too anxiety-producing on either a verbal or on a conscious level. In a drawing, the child could divulge unconscious negative attitudes by altering line quality, disguising shapes, and using signs and symbols that he or she might not be aware of. Burns (1990:ix), utilizing Rorschach's findings related to increased projective material with symmetry and the concept and centering symbols, describes the technique of family-centered circle drawings. The intent is to tap into the unconscious and uncover positive and negative energy systems through symbols.

Human figure drawings have been described as providing a means of exploring the personal constructs individuals have about themselves, others, and events in their lives (Goodnow, 1977). Drawings permit the expression of emotion through both motoric and visual representations of experience. The products allow for the expression of the association between symbols and their meanings as construed by the artist (Klepsch & Logie, 1982).

Numerous efforts to standardize drawings as a projective technique have been undertaken. The Draw-A-Person (Harris, 1963; Koppitz, 1968), House-Tree-Person (Buck, 1970; Jolles, 1971) and Kinetic Family Drawing (Burns & Kaufman, 1972), continue to be regularly introduced as part of a general psychological test battery in many evaluation settings.

Trauma Research

There is a beginning of literature on the use of children's drawings in clinical settings. Wilson and Ratekin (1990) note how nurses have found the drawings of children to be helpful as communication tools with oncology patients (Johnson & Berendts, 1986), with abused children (Kelley, 1985), as reflections of children's views of hospitalization (Allen, 1978), and as a data-collection tool regarding children's perceptions of hospital isolation (Broeder, 1985).

Children's drawings have also been used following traumatic events such as maltreatment (Lott, 1989), hospitalization (Crowl, 1980), sexual abuse, (Burgess, McCausland, & Wolbert, 1981; Hibbard, Roghmann, & Hoekelmann, 1987; Kelley, 1984), running away from home (Howe, Burgess, & McCormack, 1987), and witnessing family violence (Wohl & Kaufman, 1985). These drawings help explain the sense of distress experienced by the child, as he or she recalls the event. Juveniles incarcerated for criminal behavior are providing drawings for diagnostic and treatment planning (Burgess, Hartman, Grant, Shaw, & McFarland, 1990). Children's drawings are also being used to explain fatal illness (Fassler & McQueen, 1990).

Pynoos and Eth (1986:307-309) described their specialized interview format for use in the initial meeting with a recently traumatized child witness to violence. This format has a three stage process: opening, trauma, and closure. The format begins by permitting the child first to express the impact of the trauma in play and fantasy, and through metaphor, by use of a

projective free drawing and story-telling task. This opening enables the clinician to assess the child's preliminary means of coping and defensive maneuvers. Second, the clinician shifts attention to the actual trauma episode. In order to foster mastery of the traumatic anxiety, this overcomes the efforts of the child to avoid and deny, and supports a careful exploration of the child's experience. Finally, the clinician can then assess the child's current life concerns with an increased sense of security, competence and mastery. Although the suggested order may vary, each major step in the interview process may be depicted through a series of drawings. The entire interview generally requires approximately 90 minutes. For the free drawing and story telling, upon being seated, the child is given pencil and paper and asked to "draw whatever you'd like but something you can tell a story about." Children under 4 years of age are engaged in play along with their scribbling and are asked to "make up a story." The emphasis is for children to begin in whatever manner is most acceptable to them.

The introduction of drawings as a nonthreatening method of developing an alliance with a sexually traumatized child has been deemed useful both for purposes of lessening the trauma associated with self-disclosure, and in the subsequent treatment phase (Burgess et al., 1981). Stember (1980) described how painting the fantasies of the mind could assist in the externalization of the traumatic incident as a method to provide an avenue for healing and growth. In this way, continued Stember, the artwork functions as a vehicle for pulling together feelings and expression.

A review of the literature on graphic analysis reveals limited attention to, and a somewhat ambiguous interpretation of image characteristics, with some authors associating suspicious graphic emphases to anxiety over sexual thoughts, play, exploration, or a recent surgical procedure (to the pelvic area) (Kellogg, 1970; Kopitz, 1968; DiLeo, 1970; DiLeo, 1973; DiLeo, 1983).

Despite numerous studies which appear to attest to the clinical and diagnostic usefulness of children's drawings, it is curious that research has not advanced in the area of the use of children's drawings for diagnostic or therapeutic outcomes. The answer seems to lie in the need for independent reliability and validity of projective tests for psychodiagnosis in children. Gittleman's (1980) critical analysis of projective tests in general and drawings in particular argues that the ability to distinguish between diagnostic groups with drawings is low as are their test-retest scores.

Given this view for the projective use of drawings, our attention focused in the 1980s on the use of drawings as an associative tool in assessing and accessing stored memory at the sensory, and perceptual as well as cognitive areas (see Hartman & Burgess, 1993). The clinical research in this area hopefully will serve as pilot research in the 1990s

DRAWINGS AS AN ASSOCIATIVE TOOL FOR MEMORY

The basic premises of drawings as an associative tool for accessing memory are: (a) Children's drawings provide a view of how the child represents experiences to themselves and to others; and (b) drawings reflect cognitive structures through: the organization of the drawing, selection of content, the relationships of people and objects, and beliefs and assumptions expressed by the child about the content of the drawing.

The process of constructing memory of events derives from the interaction of information coding, storing, and communicating. The concept of emotion may be understood as being remembered through recreating physical experiences, images, and sounds. In other words, an emotion such as sadness is not identified by a person simply by the word but by the coming together of various states of bodily tension, sensation, ideation and perception. This point varies from the premises of Piaget and others (Inhelder & Piaget, 1958) who recognized

language as a higher order of information processing and thus assumed that in the more developed individual, the first level of interpreting information is at a linguistic level. Our position is that information is initially encoded, processed and organized at a sensory level and then its labeling is influenced by the higher cognitive schemas. The same is true of visual representations of past events, or for that matter, future events. The organization of an image in one's mind's eye involves kinesthetic organization, spatial identification and orientation, as well as others to name a few (Hartman & Burgess, 1993).

The Event Drawing Series

The event drawing series is a graphic presentation of the child's thinking about a specific event and can be used in several ways. First, the series may be evaluated by the individual therapist working with the child for assessment and treatment planning purposes. Second, the series may be videotaped (with the permission of the parent and child) and used for outside evaluation. Third, the drawings may be used as part of data collected for criminal and civil litigation purposes (Burgess, Hartman, Wolbert, & Grant, 1987).

The opening phase of the session, asking the child's permission to make the series of drawings and otherwise engaging the child in conversation, alerts the therapist to the child's demeanor, verbal, and nonverbal behavior, and means of defending and coping to minimize anxiety about the event. Having a task, such as drawing, helps the child to reduce the normal anxiety about talking about an emotional experience, and the drawing itself provides the therapist with valuable information.

The therapist introduces the task to the child by saying, "I would like you to do seven drawings during our time together. I will ask you to draw each drawing on a separate piece of white paper. After you finish each drawing, I will ask you a few questions about it. You may use pencil, crayon, colored pencils, or pens. Are you willing to do that?"

While the child is working on each drawing, the therapist records everything the child says and does during the drawing task. After the child finishes, he or she is asked to identify the persons, objects, and other items in the picture. Then the following questions are asked and the responses are recorded.

Drawing 1. Draw your favorite weather. After completing the drawing, the child is asked: What kind of weather is it? Why is this your favorite weather? Does this weather differ from the weather right now? If so, why?

Drawing 2. Draw a picture of your whole self (not a stick figure) as a younger child. After completing the drawing, the child is asked: How old is the younger self? What is the younger self doing? What is the younger self thinking? What is the younger self feeling?

Drawing 3. Draw a picture of your whole self as you are now. The same questions are asked as in the younger self drawing.

Drawing 4. Draw a picture of your family doing something together. The child is asked: What is each member doing, thinking and feeling? Who isn't in the picture and why? Who helps whom in the family?

Drawing 5. Draw a picture of what happened to you. The child is then asked: What is each person in the drawing doing? What are they thinking? What are they feeling? What happened to each person?

Drawing 6. Draw a picture of a house and a tree. The child is asked: Tell a story about the house (who lives in it, what is it like to live there, in what condition is it?). Similar questions about the tree include: What kind of tree is it? How old is the tree? In what condition is the tree?

Drawing 7. Make your own drawing. The child is then asked: Tell a story about your picture. The child is encouraged to be as descriptive as possible about the story.

The following briefly discusses the ways drawings are reviewed.

Drawing analysis

Realizing that only those professionals trained to interpret the artwork of sexually abused children should analyze drawings, the ways drawings are reviewed are briefly discussed.

Favorite weather. This drawing, as an introductory task, helps the child feel comfortable with the therapist and art medium, and helps the therapist evaluate the mood of the child. For example, the weather drawn may match the current weather, or may differ, possibly reflecting the child's mood or emotion.

Younger self and current self. The two drawings are compared for the child's self-representation in terms of organization, body image, and sense of self. For example, there may be no difference in size or organization, or there may be marked differences between the two selves.

Family. This drawing provides information about the child's perception of protection, safety, nurturance, and family support. It may reveal conflicts in the family and provide an estimate of support available to the child.

Event. This drawing assists memory and recall of the reported event, at the same time retrieving emotion and anxiety surrounding it. Children give their first organized level of presentation of what happened to them. There may be details and an elaborate presentation of the setting in which the abuse occurred. Some children may sequence the event by before, during, and after. Some children may only draw before or after the event. Some children will avoid or block the request (Burgess et al., 1987). Drawings represent the child's reactive capacities to the fundamental arousal stimulated by the request to relate the trauma. Further drawing and questions can bring out thought content. It can have refined visual concern such as an attribution to a mouth or the shaping of fingers; it can have auditory memories.

House and tree. The house and tree drawing is used to measure the child's organization of thinking. This drawing follows the event drawing and may illustrate intrusive thinking from the event drawing.

Free drawing. This drawing closes the interview by giving the child control and choice. It also provides a measure of the child's mood as well as evidence of continued intrusive imagery triggered by the reported event. The child's story is analyzed in relationship to its theme and relevance to the event.

EVENT DRAWING SERIES STUDIES

There are several descriptive projects on children's drawings and response patterns to sexual abuse. The following findings from the analyses of drawings are discussed.

Child Molestation

To ascertain the impact of child molestation, a series of drawings (Burgess et al., 1987) from children ages 7 and 10 were obtained prior to and following their testimony in court. Six clinicians who used art with the assessment of children independently reviewed 81 drawings and completed a drawing assessment tool that included categories on the theme of the drawing, the organization of the drawing, the age level expressed, affective responses, emotional indicators and interpretive indicators such as defense mechanisms, self representation and expression of needs. A percentage of inter-rater agreement was determined across all six raters.

The analysis of drawings indicated the following: numerous indicators of anxiety, insecurity and isolation of affect on drawings designed to elicit affect (favorite weather drawing); indicators of age regression and transparencies of self (drawings of self younger and now); presence of family conflict (kinetic family drawing); indicators of blocking, denial and avoidance (draw the event); indicators of disorganization in cognition and affect, fragmentation of body parts, repeated memory of the abuse (house & tree and & free drawing).

The findings suggest that surfacing a suppressed traumatic event for purposes of testifying in court illustrated two major phenomenon. First, when asked to recall what happened, the latent power of the event to disrupt and impair the children's performance and social interaction was noted, and second, the exploration and validation of the details of the molestation provided a window into the structural organization of personality post-trauma.

In another retrospective study, the drawings of nine children sexually abused prior to age four at a day care facility, when compared with a comparison group of eight nonabused children, the following major differences were reported: (a) avoidance of drawing details concerning the traumatizing aspects of their sexual abuse; (b) omission, shading and sexualization of body parts on drawing of self now; (c) sad or expressionless moods; and (d) themes of anxiety in all drawings. The strongest positive finding was that self-image indicators from the abused groups indicated a clear comparison between the drawings of self now and self younger, suggesting that counseling had repaired an injured sense of self (Burgess, 1988).

In this study, which evaluated the human figure drawings of 124 runaway adolescents, the relationship between the drawings of sexually abused and non-sexually abused adolescents was explored. Graphic criteria for assignment to a specific diagnostic category was developed, as well as graphic characteristics indicative of sexual concern, curiosity, and/or anxiety. Three art therapists independently evaluated the drawings. Each human figure drawing was assigned a diagnostic category. The criteria for determination of graphic characteristics which was matched with each diagnostic category, was developed through a careful assessment of the literature on graphic analysis with inclusion from many sources of those graphic descriptions which appeared to represent a consensus of agreement between more than two sources. Eleven diagnostic categories were developed; only clients who were given the same diagnosis by at least two raters were assigned to a diagnostic category. Ninety-five percent ($N = 118$) of the drawings were assigned diagnoses in this manner. Each drawing was evaluated for graphic evidence of sexual concern, curiosity and/or anxiety. As no study was available delineating image characteristics typically observed in the artwork of the sexually traumatized child, the suggested criteria for indicators was obtained by listing image descriptions mentioned by previous authors as suspicious of anxiety over sexual thoughts, play, etc., and by listing a series of hypothetical image characteristics in keeping with the exploratory nature of this study.

Four distinct psychological diagnoses were identified: psychotic, avoidant, anxious-avoidant, and anxious-aggressive. These categories were found to have statistically significant differences ($p = < 0.05$) between runaways reporting sexual abuse ($n = 53$) and those not reporting sexual abuse ($n = 50$).

From 17 graphic indicators of sexual concern, curiosity and/or anxiety, eight were identified as present in the 124 drawings. These eight were further reduced to three (based on a difference of or greater than 10%) on clients given one of three labels of anxious. These indicators were: omission lower half of body, hidden hands, and heavy hair.

This preliminary evaluation of drawings of 124 runaway youths suggests that sexually abused adolescents are more likely to be diagnosed through the use of a human figure drawing, as anxious-avoidant than their nonsexually abused counterparts (Howe, Burgess, & McCormack, 1987).

In a study of crime scene drawings of 18 juveniles who had committed murder, observations included: (a) sequencing of the crime before, during, and after the murder; (b) motivational dynamics of victim-provoked murder, victim as a physical threat, size distortion between victim and offender, displacement of rage, and drawing details (Burgess et al., 1990).

CONCLUSIONS

Drawings are useful in assisting children to discuss frightening and threatening information and perceptions about events in their lives. They give information about how memories have been stored at sensory, perceptual, and cognitive levels. The drawings provide insight into the dysregulation of memory and critical discontinuities between the child's labeling and comprehension of what had occurred. The child is presenting the direct experience, not a secondary elaboration of the experience. This appears unique to the recollection of traumatic events and is important for assessment and diagnostic purposes. It becomes the basic level of information that has to be dealt with before recovery can occur.

One promising area of research has been with juveniles who victimize other people. Moving from the details that they draw relevant to their aggressive acts, there has been association to their own earlier abuse. Fragments of victimization are noted in their criminal acts (Burgess et al., 1990). Research needs to evaluate the presentation of information and its relationship to symptomatic states as well as functional states. This would provide a method of facilitation of the child's presentation of what happened and evaluation of the effects of detailing traumatic experience in the overall behavior.

We believe that this process of drawings provides a direct way to assist the child in giving voice to the experiences that have happened to him or her and thus raises the spectrum of possibility for the child to reduce or stop living the traumatic experience and be able to reflect on the experience as a memory (van der Kolk, 1984).

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Résumé—On a recours au dessin dans des entrevues cliniques avec les enfants parce qu'il fournit des indices importants pour orienter la recherche des souvenirs. Dessiner aide à recouvrir les souvenirs des expériences motrices, visuelles et auditives. Le dessin lui-même sert à exprimer le vécu moteur sensoriel; les objets qui sont reproduits dans le dessin aident à déclencher les souvenirs; l'organisation du dessin, les dimensions interpersonnelles qui y sont représentées et le dialogue pour expliquer le dessin véhiculent les dimensions cognitives de l'enfant. La prémisse sous-jacente dans cet article c'est que les dessins des enfants sont, par rapport au processus d'association, un outil utile pour évaluer et atteindre des souvenirs traumatiques.

Resumen—Los dibujos infantiles han sido utilizados en entrevistas clínicas para obtener dirección en la recolección y memoria de los eventos. Los dibujos estimulan la recolección de experiencias en los recuerdos motores, visuales y auditivos. El dibujo en sí mismo ofrece una expresión de descarga sensorio-motora; los objetos en el dibujo ayudan con las señales perceptuales que se recuerdan; las dimensiones cognitivas se representan en la organización, patrones interpersonales y discusión verbal del cuadro. La premisa de este trabajo es que los dibujos de los niños son útiles como un instrumento asociativo para evaluar y tener acceso a recuerdos traumáticos.